

In-Class Writing Exercise

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I have Diabetes. Am I to Blame?

By Rivers Solomon

My fingertips are bruised and polka-dotted black because I am, yet again, getting back on track. A three-month bender of unbridled carbohydrate ingestion has left me a skinsack. I am made of headaches, nausea, vomiting and fatigue. After 10, 12 hours of sleep, I still need a nap because I awake hourly in the night, alternating between trips to the kitchen to guzzle diet soda, iced tea or water and trips to the restroom to urinate it all out.

I swear that this time discipline, grit and force of will — three qualities that have always seemed elusive — will reign. The glucose meter will be my new clock. My life will revolve around its numerical output. After every meal or snack, I will punch a button on the pager-size meter, setting a brief click-clack of machinery in motion before a lancet thrusts into my toughened skin. Because my fingertips have become calloused from years of this, it will sometimes take several pricks before the lancet draws enough blood to register.

Though I've done this thousands of times, I still wince at every jab. I think of medical leeches. I think of bloodletting. It is strange to live in a world where making oneself bleed is the first step to healing.

My sugar-thickened blood reminds me of unrefined petroleum. Lost in one of my many delusions, I wonder if I'm not a human but a gummed-up robot — the model discontinued because its body couldn't understand the most basic and necessary of processes: converting food into fuel.

Soon I will resume the ritual of multiple daily stabbings. I will make a shopping list full of foods I'm not particularly fond of. I'll design a workout plan to accommodate my increasingly troublesome left knee. I'll swallow pills that make my stomach and bowels spasm. I will inject insulin.

I've been diabetic for about 6 years, since age 22. Type 2, I have to add. I am young but fat, so people wonder if I have the sort of diabetes that *just happens* for no reason, typically to very young people, or if I have the sort that I brought on myself through what people perceive as a lack of willpower and self-control.

Culturally, this disease straddles the line between malignant and benign. On the one side, there's the obvious suffering — amputation, heart disease, blindness — side effects of constantly inflamed blood vessels. On the other, there's *just diet and exercise, that's all it takes*, and oral drugs and insulin. There's *you seem fine*. There's the invisibility of the deeply dedicated management it requires.

Diabetes mellitus is a class of metabolic conditions characterized by high blood sugar. The hormone insulin is the vehicle by which sugar — that much disparaged substance — enters our cells from the blood. In Type 1 diabetes, the pancreas no longer produces insulin, which means that sugar has no means to enter cells. In Type 2 diabetes, insulin resistance means that even though insulin is being produced, cells do not respond to it. While the causes are not completely understood, some combination of genetic predisposition and environmental factors including diet, exercise and stress causes the cells to need more and more insulin to be able to take up sugar from the blood. Weight and diet play a part in developing diabetes Type 2, but genetics is also a factor. As with most diseases and disorders, diabetes has a cascading effect on the body.

Every chronic illness, disease and disability carries with it misunderstandings. Too often society paints disability as a personal failing. A person with chronic pain in her legs, who is not paralyzed but chooses to use a wheelchair, may be seen as weak or lazy.

I've found my fatness compounds this phenomenon. My body is visibly off kilter, a symbol for lethargy, lack of self-regulation, ill health, indolence. Combine this with the misbelief that there is a cure for diabetes — that cure being willpower — and everyone is suddenly an expert on how to fix me. It'd be impossible not to internalize that I am to blame. There is the issue of my blackness, too, which many, because of unconscious bias, interpret as inherently lazy, deviant, sick, unclean.

I've always known my body needed transforming — or that other people thought it did. I was teased and rejected for my body throughout my years in school. I wasn't fat as a child, but I was big. Extraordinarily tall for my age (4-foot-11 in the first grade) and broad-shouldered, I might have excelled at contact sports but I wasn't built for the ballet I longed to do. I saw the attention my grandmother lavished on my skinny cousin contrasted against the frustration she expressed shopping for clothes that fit me. My mother was thankfully kind and nonjudgmental, but when I visited my father over the summers, he put me on grueling diets, including one where I couldn't eat solid foods before midday.

I had started dieting at the age of 6. My mother briefly explained calories to me because it had come up in an unrelated conversation. The next time I ate a slice of bread, I

immediately got on our family treadmill until the number on the monitor denoting calories burned matched the number of calories per slice on the package. In later years, I'd secretly drink sample bottles of perfume to try to make myself vomit.

Today, when I do manage to control my diabetes, it's at the cost of almost every other element of my life. Every bite I ingest requires a complex algorithm, calculating ratios of carb to fat to sugar to insulin to the amount of walking I've done. Even when my math is perfect, my sugars rebel. I often fall into dangerous lows (a side effect of taking too much insulin, which sends blood sugar plummeting). I eat an apple to bring my sugar up, and suddenly it's too high again.

Low-carbohydrate diets barely work for me. Even the sugar in a serving of broccoli sends my sugars to uncomfortable highs. I get anxious at parties, at restaurants out with family. Meat, potentially one of the diabetic's safest foods, is often slathered in sugary barbecue sauce or honey glaze.

I weep into my partner's arms when I realize that this level of control is not sustainable. She's been with me since I first got the diagnosis, and after the grief passed, she asked me, "What do you need me to do?" I know she's concerned about my longevity, but she doesn't put that concern before my need for a companion who's not overly invested in my every food choice.

Her gentle support isn't always enough. Diabetes demands perfection, and I am the most imperfect person I know. When eating becomes this exhausting, I simply refrain from food altogether. There is no more surefire way to blood-glucose control than starvation, and I've gone months eating only a small bowl of chicken soup a day, had doctors praise my impressive management.

The extremism with which I tackle diabetes management is directly related to the extremism I apply to food in general. A lifetime of dieting, a lifetime of being told my body is wrong, takes its toll, and I can't help conflating the messages that I am better off starved than fat. Maybe if I could let go of the shame, or more important, if the media, doctors, friends, family could stop shaming me, managing my diabetes wouldn't be this roulette wheel of self-torture. Maybe then, I could finally let go and heal.